CLASS 4

February 12, 2026

Records, Competence and Malpractice

AGENDA

- **Records and Subpoenas**
- **Competence**
  - Training and preparation
  - Credentialing
  - Maintenance and continuing education
  - Burnout
  - Diagnosis and Assessment
  - Diversity issues in testing
  - Consultation
- **Malpractice**
  - Relevant Legal cases
  - re: suicidal clients
  - re: threats to hurt others
  - reporting
- **Presenting your cases**

Records and Subpoenas

ACA ON RECORDS: WHAT STANDS OUT OR SUPRISES YOU?

- **B.6.a-B.6.h**
- B.6.a. – Counselors maintain documentation necessary for rendering professional services to their clients.
- B.6.b. – Records must be kept private/secure.
- B.6.c. – Must have permission from client to record.
- B.6.d. – Must have permission to observe sessions or review transcripts or review recordings of sessions.
- B.6.e. – Competent clients should have reasonable access to their records.
- B.6.f. – Counselors provide assistance and consultation to clients to help them interpret counseling records.
- B.6.g.. – Counselors obtain written permission from clients to disclose or transfer records to third parties.
- B.6.h. – Counselors securely store records following termination. Follow federal and state laws in disposing records keeping (**5 years in WA** per WAC 246-809-035). Be cautious about destroying records that may be needed in court (child abuse, suicide, harassment, or violence).

WHY SHOULD COUNSELORS KEEP GOOD NOTES?

- Includes plans for a case, keeps track of goals and progress, measures change and growth, affirm significant turning points
- Documents important actions/decisions. Records can serve to protect counselors. If a counselor's decision are challenged, it is critical that they have documented decisions and the action steps they have taken.
- Helps counselor remember where the session left off (especially if infrequent session occur or a break)
- Important for continuity of care if a client changes counselors
- Are primarily for a counselor's used for maximum effectiveness in treatment but keep in mind that clients may ask to see them and at some point, they could be subpoenaed. Clinicians should write note with the understanding that someone may read your notes at some later date (Herlihy & Corey, 2015
- Should be written with enough clarity for another professional to follow
- Records procedures followed in an emergency

Also, consider your setting. WHY you are taking notes. Is it primarily for your benefit? Is the client in the hospital and you are tracking progress, are other professionals likely to see the notes? Is it likely to be subpoenaed?

A FEW IMPORTANT CONSIDERATIONS REGARDING NOTE TAKING

SOAP notes (basic tool to follow, your site may have other suggestions/requirements)

- **S**ubjective-client reported…(perceptions, feelings experiences)
- **O**bjective- "counselor observed….(hand ringing, lack of eye contact, unkempt appearance"
- **A**ssessment - Making sense of S and O
- **P**lan- Action steps you took (for client and/or therapist) towards treatment goals

Or DAP (**D**ata, **A**ssessment, **P**lan)

Read Case 6-4 and write a DAP or SOAP note

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WRITING WHAT WE DID AS COUNSELORS IS AN IMPORTANT PART OF NOTES

Interventions you offered (example)
https://www.youtube.com/watch?v=a2znnlABdpg&t=147s

Have these tools ready to use in session and DOCUMENT that you utilized them in session.

OTHER RECORDS COUNSELORS KEEP

- **Administrative Records** – appt. books, billing accounts, correspondence, intake forms, and other routine papers.

- **Audio Recordings or Video Recordings of clients.**
- Records of client logging into computer information system, telephone call records, computerized or video records of client entering or leaving the facilities.

- Any other documentation made about client outside of the counseling session.

AUDIO AND VIDEO TAPES

- Often used in training and supervision (you will do this for your practicum and internship). Counselors and supervisors are responsible for:

- Getting informed consent
- After review in supervision, erase the tapes (end of each semester in clinical training).
- If labeled, do not bring attention the confidential nature of the videos
- supervisors are responsible for returning any tapes to the supervisee and handling the tapes appropriately.

Read the case of Paolo on p. 136-137

- What required information listed here may be in excess to content that is typically included?
- Would you suggest that Paolo take further steps to challenge the requirement? Why or why not?
- What step(s) can Paolo take to ensure that his record keeping is in compliance with his agency's expectations?

- What rationale might Paolo provide to his clients for requiring them to complete 12 pages of intake forms?

If a client requests their records

1. Document and get a signed document recording release of the record
2. Explain any risks of disclosure (no longer under agency protection, misinterpretation etc.), consider offering a summary of pertinent information
3. Ensure the records would not harm the client
4. Ensure other clients information is not disclosed
5. Provide assistance and consultation to review the record (consider charging a fee for this service if extensive)
6. Have a clear written policy

ACTIVITY:

WATCH THE 6.1 VIDEO AND ANSWER THE FOLLOWING QUESTIONS

1. In the video, the client states: "I know you've been keeping little notes on me to share with other people." Assuming the client was in a more calm and receptive state, what response should the counselor provide to clarify the client's misconception?

Review the client's right to confidentiality, noting that the client must provide written consent for any release of her information.

2. What is a solid rationale, rooted in counseling ethics, that supports the counselor's decision to limit a client's access to their counseling records?

Counselors may exercise discretion with sharing client records. In instances where providing full access to records would likely harm the client or damage the therapeutic alliance, it is ethically permissible for a counselor to limit a client's records. Based on the client's words and demeanor, it can be reasonably assumed that she would be harmed by gaining access to her full case file.

3. What would you say to a client who requested to view records forwarded from outside mental health professionals, such as previous counselors or psychiatrists?

If subpoenaed client records contain files and reports that were received from outside mental health professionals, counselors should request for the court to subpoena the records directly from the originating clinician.

4. Practice writing a SOAP or DAP note for this portion of the session (see p. 145-146):

Counselors must take steps to ensure client notes remain confidential. What steps are important (see case 6-2). However, you should also assume others will see your records.

THE HEALTH INSURANCE PORTABILITY AND ACCOUNTABILITY ACT (HIPAA) REQUIRES

Counselors must:

- give clients a clear written explanation of how they use, keep, and disclose their health care information.
- give clients access to their records (unless it would cause them harm)
- follow a written process for clients to request amendments to their records.
- keep a written history of disclosures of client information, available to clients

HIPPA EXCEPTIONS FOR CLIENTS THAT NEED EXTRA SUPPORT (P. 150)

- If clients are informed and given an opportunity to orally object counselors may give health case information to family members or others assisting in the client's care (consider adding this to informed consent documents)
- Counselors may provide information disaster relief organizations, such as the Red Cross.

SUBPOENAS

Subpoenas are used:

- to obtain copies of records.
- to cause potential witnesses to testify at a deposition, hearing, or trial.
- to ask potential witnesses to respond in writing to questions.
- to require witnesses to appear at a deposition, hearing, or trail and to bring their records with them.

IF YOU ARE SUBPOENAED:

- Don’t ignore it but get advice before responding. I
- Contact your client, did they receive the subpoena?
- Talk with your supervisor about obtaining legal advice or seek it yourself. Is the information requested privileged communication?
- If you are working for an agency, check with them before you hire an attorney on your own.
- **Do not just hand over records**. There are procedures lawyers must follow to give health care professionals a subpoena (see RCW 70.02.060) and the information may be privileged

- ACA Code of Ethics B.2.d. (2014) recommends counselors obtain a written release of information from their client to release records.

Record Keeping & Task Completion	**Completes *all* weekly record keeping & tasks correctly & promptly (e.g., case notes, psychosocial reports, treatment plans, supervisory report).**	Completes *all* required record keeping, documentation, and assigned tasks in a through, timely, & comprehensive fashion.	Completes *all* required record keeping, documentation, and tasks in a competent & timely fashion.

CCS-R, (E) ON RECORDS

COUNSELOR COMPETENCY

Competency

WHAT IS COUNSELOR COMPETENCY?

A complex construct with many possible levels along a continuum

- Ethical and legal considerations “do no harm”, nonmaleficence is the most salient moral principle.
- Skill considerations “perform at an acceptable level”
- Practice considerations “Within the boundaries of their competence”
- Offering ethical referrals when clients are outside counselor’s area of service (not discriminatory reasons)

Ways graduate programs attempt to ensure competent graduates:

- Selecting and admitting individuals who are likely to succeed at developing the skills, knowledge, and characteristics needed to become effective counselors.
- Using personal interviews, written essays, or other subjective criteria in selecting candidates for admission.
- Providing curriculum, feedback, and guidance for master's students that prepare students to be effective counselors.
- Requiring successful supervised experience as a part of the graduate program.
- Pointing to necessary areas of growth and offering remediation for students who fail to meet requirements
- Professors we are called to be gatekeepers for the profession.

How does the field encourage counselor competency?

LICENSURE

Professional licensure statues establish the minimum standards for practice as a licensed provider. It is the most powerful type of credentialling established by state law.

Is typically required for practice, billing insurance, practice as a clinician

Legislatures choose to license professions so that the public can determine which practitioners are competent, and to prevent harm by incompetent members of that profession.

All 50 states offer counseling licenses-reciprocity continues to be a salient issue since due to the variety in requirements from state to state but we are making progress: https://counselingcompact.org/map/

Typically needed to bill insurance

Certification (broader) also offers a way to support evidence of competency and are often a way to show areas of specially.

WASHINGTON COUNSELING LICENSES

Https://doh.Wa.Gov/licenses-permits-and-certificates/professions-new-renew-or-update/mental-health-counselor/licensing-information

LMHC-A

- education requirements (this degree)
- application

LMHC

- educational requirements
- application
- Passing score on NCE or NCMHCE Exam
- 3000 hours (1200 direct,100 supervision hours) or 3 years in full time work as a counselor

.

CERTIFICATION

Certification (broader) also offers a way to support evidence of competency and are often a way to show areas of specially.

- Counselors can offer evidence that supports their competence by becoming certified
- Various types of counselor certifications exist
- Some types of certification are mandatory for practicing in certain settings and occur at the state level (e.g., public school counselors must be certified by the states in which they practice)
- There are also national, voluntary certifications, such as those offered by the National Board of Certified Counselors
- Specialty certifications are another means of establishing a counselor's competence to work with certain types of clients, in certain settings, or in specialized areas of counseling.
- Specialty certifications are offered by various groups in such areas as hypnotherapy, biofeedback, and sex therapy

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Once counselors begin to practice, they are responsible for determining their own competence.

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Maintaining Competence (1 of 4)

- **Continuing Education:**
  - The ACA *Code of Ethics* (2014) states that counselors recognize the need for continuing education to acquire and maintain awareness of current scientific and professional information.
  - Most counselor licensure and certification boards have established continuing education requirements for maintaining one's license.

Maintaining Competence (2 of 4)

- **Peer Review/Peer Consultation:**
  - Useful for counselors at all levels of experience and should continue as a lifelong learning process.
  - Benefits include enhanced self-awareness, mutual support, objective feedback in dealing with countertransference issues, information on therapeutic techniques, and/or help in dealing with difficult cases, stress, and isolation.

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Maintaining Competence (3 of 4)

- **Technology:**
  - The internet presents counselors with myriad opportunities for keeping current and improving their competence as practitioners.

Maintaining Competence (4 of 4)

- **Making Referrals:**
  - The ACA *Code of Ethics* (2014) states that counselors avoid entering into a therapeutic relationship when they lack the competence to counsel a prospective client.
  - Ethical counselors recognize that they will need to refer a client when accepting or continuing to work with that client would exceed their boundaries of competence.
  - Referrals should be based on competence and rather than discrimination.

Erosion of Competence (1 of 3)

- **Burnout and Impairment:**
  - The work of the counselor can be stressful and emotionally demanding
  - Most counselors could probably be described as *distressed* at some time during their professional lives
  - Unalleviated distress can lead to *burnout*
  - Burned-out counselors have little energy left for their clients
  - Common indicators of burnout include decline in self-care, strain in personal relationships, and damage to overall emotional health

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Erosion of Competence (2 of 3)

- Impaired counselors are unable to perform their responsibilities appropriately, thus, putting their clients at risk.
- Impairment is often associated with alcohol and other drug abuse and with the blurring of therapeutic boundaries that can lead to sexual exploitation of clients.
- Risk factors for impairment include:
  - environmental factors
  - vicarious traumatization
  - preexisting conditions

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Erosion of Competence (3 of 3)

- Corey et al. (2024) have suggested that counselors ask themselves some of the following questions if they think they might be experiencing burnout:
  - Do you ask supervisors or peers for help when you need it?
  - Are you willing to seek personal counseling if you think you need it?
  - Do you engage regularly in self-care practices?
  - Are you engaged in life outside of work?
  - Are you staying connected to family members and friends?

2014 ACA codes on impairment

- Read sections C.2.g and F.5.b and identify the following:
  What are counselors, students and supervises asked to monitor in themselves?
  What actions are they asked to take?
- What signs of mild, moderate and severe impairment would you need to look for in yourself? What steps would you take at each level?
- What would you do if you noticed a supervisor or colleague showing signs of impairment?

If counselors determine they are impaired, they must limit, suspend, or terminate their professional responsibilities and seek assistance to address their problems.

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CHRISTIAN CHARACTER VIRTUES, WATSON, 2018

Character virtue	Professional Ethic	Supervision models	Spiritual disciplines
Faith	Fidelity	Alliance & attachment	Cont. Prayer & Church
Hope	Beneficence	Vocational mentoring	Sabbath & Worship
Love	Respect/autonomy	Relational Competence	Intercessory prayer
Justice	Justice	Diversity Competence	Hospitality & Service
Temperance	Nonmaleficence	Reflective practice	Examen & Fasting
Courage	Integrity/veracity	Wellness & Resilience	Rule of life & Example
Practical wisdom	**Competence**	**Competency based**	**Scripture & discern.**

TO CULTIVATE WISDOM (WATSON, 2018)

Dallas Willard in his book on spiritual disciplines writes, "*Study* as a spiritual discipline is the focusing of the mind upon God's works and words. In study our mind takes on the order in the object studied, and order invariably forms the mind itself and thereby the soul and the life arising out of it." (1998)

"Regular study of God's Word informs our inward and outward habits of thinking and action towards God's kingdom" (p. 117).

What do you study to gain wisdom? Who do you respect? Where do you go to gain wisdom?

What habits do have to cultivate wisdom?

DISCERNMENT (WATSON, 2018)

Ignatius of Loyola, founder of the Jesuits from the 1500s, addressed discernment as a habit, a way of seeing that eventually permeates life... He defined the aim of discernment as "*finding God in all things in order that we might love and serve God in all*"

Ruth Haley Barton (2008) explains that prayerful discernment includes:

- Collecting and considering data
- Inner reflection and confirmation
- Action
- Submission to God's will and guidance

Competence for Terri Watson is supported spiritually by seeking God’s wisdom through scripture reading and practicing discernment.

What **practices** will be important for you to take to help you to act competently in the face of inevitable challenges? What are the risks you might encounter that could test your ability to make competent decisions in counseling?

Intakes: video 7.1

- In the video, the counselor learns from the client's intake forms that she is a single mother who recently finalized a divorce. Yet, the counselor states: "Coming in today, did you have anything you wanted to talk about? Does the counselor's decision to not immediately inquire about the client's divorce illustrate an irresponsible approach to the intake interview? Why, or why not?

- How does the counselor utilize the concept of birth order in a manner that is strengths-based and respects his client's unique identity, rather than reducing her personality and attitudes to a narrow label or a confined category?

Assessment

- Counselors have the education and skills necessary to conduct certain types of evaluations.
- Some counselors have jobs in which they conduct evaluations on a regular basis
- Some counselors have private practices in which they conduct evaluations occasionally or exclusively.
- To conduct an evaluation, a counselor should have expertise in the area that is being evaluated.

Formal Evaluations

- Clients who are being assessed need to understand that:
  - the process is different from counseling
  - what the process entails
  - what the final product will be
  - how and by whom the assessment is intended to be used.
- According to the ACA *Code of Ethics* (2014), the client's *written* informed consent should be secured before beginning a forensic evaluation (ACA, 2014).

Counselor as Expert Witness

- Counselors should avoid conducting evaluations that cast them in the role of expert witness unless they have agreed to do so.
- If your job requires that you conduct evaluations, or if you complete evaluations in your private practice, you must be prepared to testify in court as an expert witness.
- There is a good chance that eventually you will be subpoenaed and ordered to explain and defend your conclusions and recommendations.

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Testing

- Counselors use tests in various ways.
- Counselors typically select existing tests, administer them, and interpret them for their clients.
- Competence to use a test always depends on the individual counselor's training and experience with the particular test.
- Clients have the right to make an informed choice about whether they want to participate or refuse to engage in testing procedures.
- Clients can also expect to receive an explanation of the results of any test they take.

Diagnosis (1 of 2)

- Counselors are sometimes uncomfortable with participating in the diagnosis of mental disorders.
- Diagnoses usually are made using the *DSM-5-TR* (American Psychiatric Association, 2022).
- This system of diagnosis is based on the medical model of mental illness.
- When clients want to use their health insurance to help pay for mental health care, counselors typically must assign a diagnosis for their services to be reimbursable.

Diagnosis (2 of 2)

- Counselors must be knowledgeable about laws that affect testing practices.
- Clients must understand the implications of any diagnosis that might be assigned to them.
- Counselors must be prepared to work cooperatively with physicians when:
  - clients may have a physical condition that is contributing to their mental or emotional problems
  - clients could benefit from taking medications for their condition

Video 7.2

In the video, the client is deeply troubled by the provisional diagnosis he receives. Imagine that the counselor could have anticipated the impact that this diagnosis has had on her client and, instead, chose to refrain from rendering a diagnosis. Would her decision—based on a desire to prevent causing her client distress—represent an ethical approach? Why, or why not?

- In the video, the client expresses concern about his diagnosis being reported to his insurance company, due to the stigma he associates with bipolar disorder. Because the client needs a diagnosis in order to receive health insurance reimbursement, is it ethical for the counselor to code a diagnosis that the client feels is less-stigmatized, but also reflects similar diagnostic criteria? Why, or why not?

Your cases!
(Isaac, Vijay, Emmanuel, Martina

Lorenzo, Hope, Marcie

MALPRACTICE AND RESOLVING LEGAL AND ETHICAL CHALLENGES

Malpractice (1 of 3)

- *Malpractice* involves professional misconduct or unreasonable lack of skill.
  - Has been defined as the failure to render professional services to the level expected in a particular community by a prudent member of the profession with the result that a recipient of services is harmed.

Malpractice

- In order for a client plaintiff to prevail in a malpractice lawsuit against a counselor, the plaintiff must prove the following elements:
  - The counselor had a duty to the client to use reasonable care in providing counseling services
  - The counselor failed to conform to the required duty of care
  - The client was injured
  - There was a reasonably close causal connection between the conduct of the counselor and the resulting injury (known as *proximate cause*)
  - The client suffered an actual loss or was damaged

Most common reasons for malpractice lawsuits

- Mental health professionals are sued most often because of sexual relationships with their clients.
  - One of the biggest payouts to individuals who sued counselors have to do with counselors engaging in sexual relationships with clients.
  - According to WA state law a counselor may never engage or attempt to engage in a sexual relationship with a **client OR former client** (see WAC 246-16- 100). This is MORE restrictive than the ACA code of ethics
- The next leading cause of malpractice lawsuits against counselors revolves around situations in which clients attempt or complete suicide.

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If a formal ethics complaint is filed against you, you should:

- respond fully and in writing to each of the charges made.
- consult with an attorney.
- submit as much documentation as you can that will help the ethics committee in its deliberations.
- contact your professional liability insurance carrier

Do not contact the client who filed the complaint to see if you can work things out.

video 8.1

- In the video, the prosecutor calls the counselor's competency into question. How does the counselor's referencing of his clinical records strengthen the credibility and clinical justifications for discontinuing counseling with the client who is suing him?

- The counselor indicates that he sought consultation for his client's issues on two separate occasions. Do the counselor's efforts to gain outside perspectives and insight from other experienced clinicians support the client's accusation of malpractice? Why, or why not?

When a client threatens to commit suicide, an ethical duty arises to protect the client from harm to self.

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Clients at Risk for Suicide

- Confidentiality requirements are waived when disclosure is necessary to protect clients or others from *serious and foreseeable harm* (ACA, 2014, *Code of Ethics*, §B.2.a.).
- The counselor's first responsibility is to determine that a particular client may be in danger of attempting suicide.
  - Evaluating and managing suicide risk is one of the most stressful situations that you will encounter in your work.
  - Counselors should focus on assessing client protective factors against suicide while also assessing client risk factors for suicide.

Clients at Risk for Suicide

- It is necessary for all counselors to know the warning signs that a particular person is at risk for suicide.
- Counselors have a legal duty to evaluate accurately a client's potential for suicide.
  - Counselors can be held liable for overreacting and for underreacting.
  - However, courts have generally been reluctant to hold counselors accountable for harm that results from clients who attempt or complete suicide.
  - Some counselors require clients at risk for suicide submit to an evaluation by a mental health professional who has an expertise in suicide as a condition for continuing to provide counseling.

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Making Assessments for Suicide Risk

- If you work in an agency, school, hospital, or other organizational setting, you must always follow procedures established by your employer.

- If your employer does not have guidelines for how to manage potentially suicidal clients, you should ask that guidelines be established for the welfare of clients and to protect the counselors who have to make decisions in these difficult situations.

Non-suicidal self injury

- In recent years, the professional counseling literature has introduced considerable discussion regarding *nonsuicidal self-injury*
  - such behaviors involve intentional bodily harm that cause tissue damage without an individual having the intent to die
- The prevailing belief among mental health professionals is that self-injury is not automatically a predictor of suicide attempts.
  - self-injury and suicidal ideation are two separate behaviors and therefore should be assessed separately

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How do you determine if a client is suicidal?

- Do you have a plan?
- Can you tell me specifically what you plan to do?
- Do you have access to the weapons, pills, etc., that you plan to use to end your life?
- Have you ever attempted to take your life before? If yes, what happened

Guidelines for assessing suicidal behavior

- Take direct verbal warnings seriously
- Pay attention to previous suicide attempts
- Identify clients suffering from depression
- Be alert for feelings of hopelessness and helplessness (giving up prized belongings)
- Monitor severe anxiety and panic attacks
- Determine whether individual has a plan
- Identify clients who have a history of severe alcohol or drug abuse
- Be alert to client behaviors (e.g. giving prized possessions away, finalizing business affairs, or revising wills)
- Determine history of psychiatric treatment

Clients Who May Be at Risk for Harming Others

- Making this decision as to whether a client may be violent in the future is difficult, as there is no scientific certainty for such decisions.
- Standard B.2.a. of the *Code of Ethics* (ACA, 2014) states that the counselor's confidentiality requirement "does not apply when disclosure is required to protect clients or identified others from serious and foreseeable harm."
- If you do determine that a client may be a danger to another person, the law requires that you take whatever steps are necessary to prevent the harm and protect the foreseeable victim from danger (Terasoff case)

Clients Who May Be at Risk for Harming Others

- Choices range from the least intrusive action (obtaining a promise from your client not to harm anyone else) to the most intrusive (having the client involuntarily committed to a psychiatric facility).
- Numerous possibilities exist between these two extremes, such as:
  - notifying the client's family members and getting one of them to take responsibility for keeping the client under control
  - persuading the client to be voluntarily committed to a residential facility
  - notifying the police
  - calling the client periodically

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What should you do if an angry client threatens to sue you?

- respond directly to the person making the threat, if possible.
- Calm the person down, if possible, and listen to the concerns.
- Be careful and not admit to wrongdoing.
- Do not say anything that you would not want repeated or would not want to repeat under oath at a later time.

The cases of Monica and the case of Javier

1. Review the cases of Monica (p.202) and Jovier (p.207) in pairs and discuss action steps you would take. Then take turns role playing the counselor and client for these two cases.